

ClearPath

AI-Powered Mental Health Triage and Provider-Match Platform

From "I need help" to the right provider —
in minutes, not weeks.



Bilisuma Tadesse | June 4, 2026

The System Fails People Who Are Already Trying

29.5 Million

Americans with diagnosed mental illness received no treatment in 2024.

48 Days

Average wait for a first appointment at community mental health centers.

The Access Gap

No structured way to understand care needs.

Only ~18.5% of psychiatrists are accepting new patients.

1 in 3 privately insured are unable to find in-network care.

Insurance costs are often unknown until after the first visit.

People abandon the process—not because they don't want help, but because the system provides no intelligent path forward.

Who Is Being Left Behind



Primary User:

- Age 25–45, employed, insured.
- Experiencing mild-to-moderate anxiety or depression.
- Has already tried to find help — and hit walls.
- Needs fast, personalized guidance, not generic information.
- Reachable through their employer's benefit program.

Why It Matters:

- This is the largest, most commercially reachable underserved population.
- Their untreated condition is the primary driver of measurable workplace productivity loss.

**This is not a crisis intervention product.
It is the layer that prevents crises from developing.**

Who Is Affected and What Each Party Needs

Stakeholder	Core Need	What ClearPath Addresses
Patients / Employees	Fast, matched, affordable care	Eliminates unstructured search; surfaces the right provider in minutes
Employers / HR	Productive workforce; measurable benefit ROI	Connects employees to care before productivity loss compounds
Health Insurers	Reduced avoidable ED visits; parity compliance	Provides structured triage evidence and step-appropriate routing
Therapists / Providers	Full caseloads with well-matched patients	Pre-qualified referrals matched by clinical need and insurance
Regulators (FDA)	Non-diagnostic scope; validated instruments	Screening tool, deterministic scoring; no clinical claims

Key System Constraints

- **Screening, not Diagnosis**
 - Diagnosis and treatment are deferred to licensed providers.
 - Regulatory posture: FDA General Wellness.
 - No PHI stored in MVP1.

What ClearPath Builds First — and What It Does Not

The Core Loop (Built in 7 Days)

1. Profile intake (ZIP, insurance, concern)
2. Structured PHQ-9/GAD-7 screening
3. **Deterministic** scoring
4. Stepped-care recommendation mapping
5. Ranked provider shortlist generation
6. Plain-language results summary

Intentionally Excluded in MVP1:

- Real-time availability data
- User accounts
- Live insurance checks
- Booking integration
- HIPAA architecture

Key Tradeoffs:

- Speed vs. Real Data: Seeded provider data proves the match loop.
- Safety vs. Completeness: Crisis detection is non-negotiable; cosmetic features are cut.
- Scope vs. Quality: Features are cut, not corners.

AI as a Precision Triage Engine — Not a Chatbot

What AI Does (Language / Generative)

- Journey guidance and context-aware framing.
- Empathetic context personalization.
- Severity-sensitive matching profile generation.
- Real-time crisis language detection (supplemental).

What AI Never Does (Clinical / Deterministic)

- Computes PHQ-9 or GAD-7 totals.
- Assigns a severity band or care level.
- Makes a diagnostic claim.
- Provides therapy or clinical advice.

The Safety Guarantee: All clinical scoring is deterministic code. The LLM receives language tasks only. This eliminates clinical hallucination risk from every output.

A Buyer-Present Market with Measurable ROI

Phase 1 (Months 1–6):
Employer B2B SaaS (\$3–8 PEPM).
Target: Mid-market (200–5K employees).

Phase 2 (Months 6–18):
Insurer / Health Plan (PMPM).

Phase 3 (Months 18–36):
Provider Network placement.

The Math: Why Employers Pay

\$47.6 Billion/year U.S. loss in unplanned absences due to poor mental health.

The 5:1 ROI Equation:

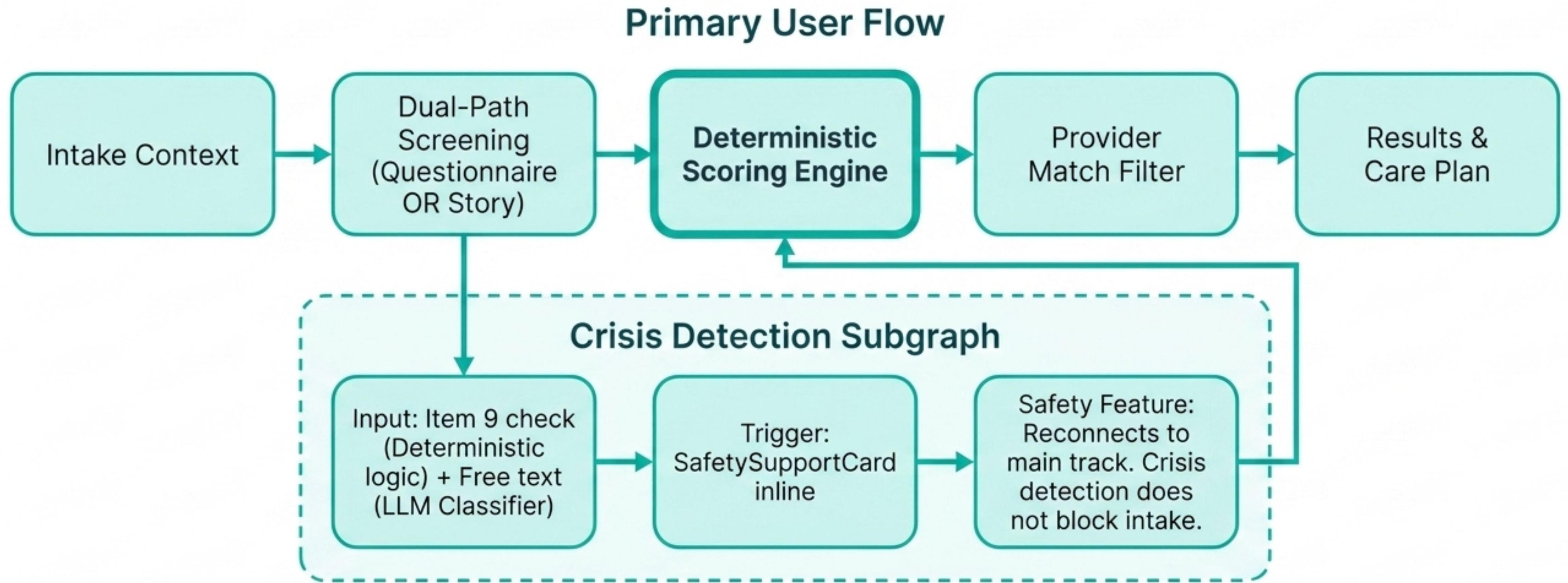
One 500-person company (\$65K avg. salary) → 5% productivity recovery

- ~\$163,000/year recovered
- ~\$30,000/year ClearPath cost

~5:1 ROI

Competitive Gap: Mid-market employers lack a validated triage product at accessible price points—ClearPath fills this specific void.

How ClearPath Works End to End



What One Week Proved

Live Prototype: <https://clearpathcare-demo.vercel.app>

GitHub: github.com/bilitade/clearpath

The Breakthrough: The core loop works. From zero context to a ranked provider shortlist — safely, in under 7 minutes.

Live Features Shipped:

- ✓ Profile intake & validated screening.
- ✓ Deterministic scoring & stepped-care recommendation.
- ✓ Dual-layer crisis detection with non-blocking safety UX.
- ✓ AI-generated plain-language summaries with deterministic fallbacks.

Stack: Next.js + TypeScript, Tailwind CSS, Vitest (50 tests), Vercel.

From Proof of Concept to Pilot-Ready

30-Day Timeline



Week 2: Make the Data Real (NPPES registry integration).



Week 3: Clinical Sign-Off (2-3 licensed clinicians review pacing and safety).



Week 4: Land the First Employer (Sign first 200+ employee pilot).



Months 2-4: Close the loop (Live insurance verification, booking handoff).

Success Metrics at Day 30

Metric	Target
Intake completion rate	> 65%
Provider match click-through	> 35%
Real provider coverage	≥ 500 providers, 5 metros
Clinical advisory validation	Complete
Employer pilots signed	≥ 1
Time-to-first-appointment vs. baseline	Measurably below 48 days

The Right Problem, the Right Scope, the Right Time

The Insight

The system lacks an intelligent layer to convert the intent to seek help into actual matched care.

Proven in 7 Days

A user goes from intake to a matched shortlist safely in under 7 minutes, with zero diagnostic claims.

Market Opportunity

29.5M untreated Americans; active HR budgets fighting massive absence costs.

Path Forward

Concrete scaling. One 500-person pilot = \$2,500 MRR. Core loop infrastructure is foundational, not throwaway.

ClearPath: from weeks to minutes. Safely.

Explore ClearPath

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